



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : رضا على صالح محمد
المحافظة : الجيزة
الحى / المدينة : ابوالنمرس
رقم البطاقة : 28311092102425 رقم التليفون : 01122041613
العنوان : ..
العمر (تاريخ الميلاد) : 09-11-1983 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 05-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج كيماوي تاكسان (دوسيتاكل) قسم العلاج :
[14152]
الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: رضا على صالح محمد National ID: 28311092102425	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Docetaxel chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



الرجاء فتح ملف
١٨/١٠/٢٠٢٤
مستور
١٤
١٤

١٨٦٨
٢٠٢٤



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
طبية
مختصة

06/30/2026

MDT FORM

Name: رضا علي صالح محمد	MRN: 11002
Age: 43 Y	ID: 28311092102425
Menopausal Sta: PRE	Phone: 01122041613
NRS SCORE:	Marital Status:
BMI:	Family History: -VE
CO Morbidity: M.FREE	Reason Of MDT: NEW CASE

Clinical Data:

BX: RT.IDC G3 //LT. FIBROADENOSIS

ER: 0/8 **PR:** 0/8 **HER2:** 0 **KI67:** 40%

R

L

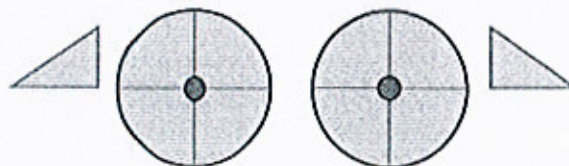
Sonomammography:

ACR C
RT. UOQ AT V10 OCLOCK ZONE 2 3.3*3/2 MASS CONTACTING OVERLYING SKIN
SKIN THICKNING 4MM
MULTIPLE SUSPICIOUS RT AXILLARY LN
ONE SUSPICIOUS RT. SUPRA CALVICULAR LN BIRADS5

LT HIGHLY SUSPICIOUS UOQ ASYMMETRY BIRADS 4A>>BIOPSIED >>>

Mets Workup:

CT >>> PULMONARY NODULES (less than 0.7 cm)



TNM

History:

Recommendations:

MDT Outcome & Plan

CTs reviewed and there are 3 small pulmonary nodules (all 0.5-0.7 cm)

For:

Port-cath insertion// Bone Scan and ECHO.

start NACT according to Keynote 522 Protocol:

Carbo -Taxol- keytruda then AC- Keytruda * 4

then re-assess for surgery according to response.

BRCA test

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد عبدالعزيز	د/ محمد النوراني				

Name: رضا على صالح محمد		GUSTAVE ROUSSY	
MR. : 11002			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 37-year-old individual with a newly diagnosed right breast carcinoma staged clinically as cT4b N3 M0. Imaging demonstrates a right breast mass measuring 33 × 32 mm with skin involvement (skin thickness 4 mm) and multiple suspicious level 1–3 right axillary lymph nodes, the largest 24 × 13 mm, as well as a right supraclavicular node measuring 10 × 5 mm. Staging CT of the chest, abdomen, and pelvis reveals three small pulmonary nodules ranging from 0.5 to 0.7 cm. A port-catheter has been placed, and bone scan, free-fluid study, and echocardiogram have been completed. Baseline laboratory work including CBC, chemistry, TSH, free T3, and free T4 has been obtained, with results showing a white blood cell count of $8.1 \times 10^9/L$ and hemoglobin of 16.0 g/dL. The treatment plan is to initiate neoadjuvant chemotherapy per the KEYNOTE-522 protocol, beginning with carboplatin, paclitaxel, and pembrolizumab, followed by doxorubicin-cyclophosphamide with pembrolizumab for four cycles, with reassessment for surgery based on response. BRCA testing has been ordered. The subsequent plan calls for administration of week 2 taxane-carboplatin (TC) together with cycle 1 pembrolizumab, pending approval and availability, with pharmacy coordination in week 3 and laboratory work in week 4. The patient has therefore completed week 2 of TC and is currently receiving cycle 1 of pembrolizumab.			
تحريراً في ,			
التاريخ : 18-09-2026		الوقت :	
هنا أسامه عبد الرحيم عبد البر		الطبيب المعالج :	
يعتمد ,			



Patient's Name: REDA ALI SALEH
Date: 2026-04-26
Referred by: Prof.Dr.
Hospital: Goustave Roussey International Egypt

BIATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

- Breast density: Moderte dense glandular tissue... Matching ACR C
- Thickened right breast skin associated with underlying irregular shaped mass with speculated margin is seen in the upper outer quadrant of the right breast.
- Focal assymetry is seen in the left breast .
- Normal skin and nipple on left side .
- Right axillary dense LNs .

By complementary breast ultrasound:

- Homogenous (predominantly glandular) echo-texture of both breasts.

Right breast :

- An irregular shaped non circumscribed margin hypoechoic mass is seen in the right breast at 10 o'clock position, zone 2 .it measures (33 x 32 mm) the mass is seen contacting the overlying skin.(skin thickness 4 mm).
- Multiple suspicious level 1,2 and 3 right axillary Lns the largest measures (24 x 13 mm).
- One suspicious right supraclavicular LNs measuring (10 x 5 mm).

Left breast :

- Corresponding to the left UOQ asymmetry :There is area of heterogenous parenchyma is seen close to 1 o'clock position ,zone 2.
- Non specific axillary lymph nodes .

CONCLUSION:

- Right breast :Highly suspicious UOQ breast mass associated with heavy nodal affection for biopsy BI-RADS 5 category.
- Left breast : UOQ focal assymetry for post contrast study BI-RADS 4A category.

Prof.DR.MONA ZAKY

Patient Name	: الأستاذ / رضا على صالح محمد	Request No.	: 13006269
Patient No.	: 28311092102425	Request Date	: 04/05/2026
Path No.	: 1295/26	Verification Date	: 17/05/2026
Sex	: Female	Financial Category	: جوستاف رومى - مبادره
Age	: 42 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast UOQ highly suspicious mass (BIRADs 5) and left breast UOQ (BIRADs 4a) area of heterogenicity.

Nature of Specimen:

- 1- Right breast UOQ mass, Tru-cut biopsy, collected on 04/05/2026, cold ischemia time: 20 seconds.
- 2- Left breast lesion, Tru-cut biopsy, collected on 04/05/2026, at 03:30 pm, cold ischemia time: 20 seconds.

Gross Examination:

- 1- Right breast mass:
 - Three grey white tissue cores, the largest is 1.3 cm, totally submitted.
- 2- Left breast area:
 - Three grey white tissue cores, the largest is 1.3 cm, totally submitted.

Microscopic Examination:

- 1- Right breast mass:
 - * Histological type: Invasive duct carcinoma.
 - * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2)
 - * Ductal carcinoma in situ: Absent.
 - * Angioinvasion: Absent.
 - * Perineural invasion: Absent.
 - * Necrosis: Present.
- 2- Left breast:
 - Histopathological examination revealed cores of breast tissue showing fibroadenosis with no evidence of malignancy.

Diagnosis:

Verified by :
Dr.Duaa Mohamed



Patient Name : الأستاذ / رضا على صالح محمد	Request No. : 13006269
Patient No. : 28311092102425	Request Date : 04/05/2026
Path No. : 1295/26	Verification Date : 17/05/2026
Sex : Female	Financial Category : جوماتاف رومي - مبادره
Age : 42 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

1- Right breast mass, Tru cut biopsy:
Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2 and Ki67).

2- Left breast area, Tru-cut biopsy:
Consistent with fibroadenosis.

Signature
Dr. Walaa Ibrahim Ghoneim.

Verified by :
Dr.Duaa Mohamed



Patient Name	: الأستاذ / رضا على صالح محمد	Request No.	: 13006269
Patient No.	: 28311092102425	Request Date	: 04/05/2026
Path No.	: 1295/26	Verification Date	: 17/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 42 Y	Department	: Out Patient
Referreing Doc. :		IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass "diagnosed as invasive duct carcinoma grade 3".
Nature of Specimen: Tru-cut biopsy, collected on 04/05/2026, cold ischemia time: 20 seconds.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67%: 40%.

Conclusion:

Right breast mass, Tru-cut biopsy:

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 40%

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between

Verified by :

Dr.Duaa Mohamed



Patient Name	الأستاذة / رضا على صالح محمد	Request No.	13006269
Patient No.	28311092102425	Request Date	04/05/2026
Path No.	1295/26	Verification Date	17/05/2026
Sex	Female	Financial Category	جوستاف رومسي - مبادره
Age	42 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim

Verified by :
Dr.Duaa Mohamed

Printing Date : 17/05/2026 12:37:38

Page 2 of 2

Printed By : .marwa mohamed yousse





Patient's Name: REDA ALI SALEH
Date: 2026-04-26
Referred by: Prof.Dr.
Hospital: Goustave Roussey International Egypt

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Prof.DR.MONA ZAKY

Patient's Name: Reda Alimed

Date: 2026-06-01

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt

- ☐ No intra or extra hepatic biliary radicles dilatation.
- ☐ Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- ☐ Normal CT appearance of the spleen with small splenules .
- ☐ Regular filling of the urinary bladder showing no stones, masses .
- ☐ No retro-crural, para-aortic or pelvic lymphadenopathy.
- ☐ No ascites.
- ☐ No worrisome lytic or sclerotic bony lesions .
- ☐ No vertebral structural collapse .

OPINION:.

- Right breast mass
- Few enlarged right axillary lymph nodes
- Multiple bilateral pulmonary nodules

Dr.Reham Khedr, MD

Patient's Name:

Reda Al

Date:

2026-06-01

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported right breast carcinoma.

(I) Chest:

- ☐ Right breast mass ROQ measures 4.2 x 2.4 cm
- ☐ Few enlarged right axillary lymph nodes , the largest 18 x 14 mm.
- ☐ Multiple bilateral pulmonary nodules , the largest seen at right upper lobe anterior segment measures 9mm.
- ☐ Few enlarged mediastinal lymph nodes , the largest at subcarinal group measures 9 x 7 mm.
- ☐ Clear both lung fields .no sizable pulmonary masses or consolidation patches
- ☐ No pleural or pericardial collections seen.
- ☐ No gross cardiac abnormality.

=

(II) Abdomen and pelvis:

- ☐ The liver is enlarged in size showing homogenous CT density ,no focal lesion
- ☐ Patent and homogenously enhancing portal vein.

PATIENT NAME رضا علي صالح			Registered 30-08-2026 19:35:35	Collected 30-08-2026 19:36:03
			Authenticated 19-09-2026 16:24:25	Printed 26-09-2026 20:26:54
Visit Number 62226506017	Age 43 Year	Gender Female	Referred By Prof : -	Client ID 55452

negative or false positive results.

- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical findings, family history, other lab investigations, and international guidelines for treatment.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.
- Richards, Sue, et al. "Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology." Genetics in Medicine 17.5 (2015): 405-423.

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University 2-2

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Visit Number 62226506017			Authenticated 19-09-2026 16:24:25	Printed 26-09-2026 20:26:54
Age 43 Year	Gender Female	Referred By Prof : -	Client ID 55452	

MOLECULAR BIOLOGY REPORT

BRCA 1.2 WHOLE GENE SEQUANCING / BLOOD (Astrazneca)

Result : No Pathogenic variants detected in BRCA1 or BRCA2 genes sequencing.

Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the BRCA1 & BRCA2 genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.
- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.
- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, BRCA2 carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRaCA assay
- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of BRCA1 & BRCA2 genes.
- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome (Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.
- Benign and likely benign variants are not included in this report.
- Variants of unknown significance classification may change over time.

Limitations

- Sanger sequencing of the detected variant is recommended.
- Large rearrangements may not be detected, variants in regions with high GC content or pseudogenes may give false

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University

1-2